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Name: اميره عبدالنواب علي سيد National ID: 28609182101265	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

اميره
عبد القواب على سيد احمد
قرية العزيزيه
مركز البدرشين - الجيزه

١٩٨٩/١١/٢٨ ٢٨٦.٩١٨٢١.١٢٦٥
J18497531

٢٠٢٢/٠٥ ٢٨٦.٩١٨٢١.١٢٦٥
ربة منزل
أنثى
مسلمة
سلامه مطاوع الويكر محمد
البطاقة سارية حتى ٢٠٢٩/٠٥/٠٦

Patient Name: Amira Abd Al Tawwab Ali	Date : 03/11/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 39 Year(s)
Medical ID: 1034/2025	Pat. ID: 99267

BREAST HIGH RESOLUTION ULTRASONOGRAPHY REVEALED:

Indication: Follow up after neoadjuvant chemotherapy of pathologically proved left breast carcinoma.

- Homogenous fatty background parenchyma.
- The previously described malignant mass seen at 1 o'clock position of the left breast ,zone 1-2 ,now measures (13 x 12 mm) and the other similar one at 1 oclock position zone 1-2 now measures (8.5 x 5.5 mm) .
- No other suspicious masses in other quadrants.
- No detected suspicious masses in the right breast .
- Normal skin, subcutaneous fat and nipple areola complex on both sides.
- No significant axillary lymph nodes.

Conclusion:

- Left breast (BIRARDS 6 category).
- Normal study of the right breast (BIRADS 1 category).

Dr farah elshobaky

Patient Name : الأستاذ / اميره عبد التواب على سيد
Patient No. : 28609182101265
Path No. : 865/26
Sex : Female
Age : 39 Y
Referring Doc :

Request No. : 13005685
Request Date : 02/04/2026
Verification Date : 07/04/2026
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- Ductal carcinoma in situ: Absent.
- Nipple & areola: Free.
- Deep margin: Free.
- Axillary lymph nodes: Fourteen lymph nodes free of viable metastatic deposits, LNs (0/14) (Three lymph nodes showing therapy effect).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0.

Signature:

Dr. Walaa Ibrahim Ghoneim.

Dr. Duaa Mohamed.

Verified by :

Dr.Duaa Mohamed

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

٢٥٢

Patient Name : الأستاذه / اميره عبد التواب على سيد
Patient No. : 28609182101265
Path No. : 865/26
Sex : Female
Age : 39 Y
Referring Doc :

Request No. : 13005685
Request Date : 02/04/2026
Verification Date : 07/04/2026
Financial Category : جوستاف روسي - افقة الدولة
Department : Out Patient
Registered by : Mrs.Mâyada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.
Nature of specimen: Left modified radical mastectomy.

Gross Examination:

- * Left modified radical mastectomy specimen measured 22x20x5 cm, with covering skin ellipse measured 20x17 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site 8x7x3 cm, located 2 cm from the least deep margin.
- * Axillary fat measured 8x6x2 cm, entangling 14 lymph nodes, the largest measured 1.5x1x0.5 cm.
- * 12 Blocks:
1:6- T.B.S., 7- N&A., 8- Deep margin., 9:12- ALNs.
- Gross was done by; Dr. Heba Hassan.

Microscopic Examination:

- * Histological type: No residual viable remnants of the previously diagnosed invasive tumor.
- Cellularity (0%).
- * Angioinvasion: Absent.
- * Ductal carcinoma in situ: Absent.
- * Additional findings: Effect of therapy (Hyalinosis & microcalcification).
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Fourteen lymph nodes free of viable metastatic deposits, LNs (0/14) (Three lymph nodes showing therapy effect).
- * Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:

Left breast cancer (Post-NACT), Left modified radical mastectomy:
- No residual viable remnants of the previously diagnosed invasive tumor.

Verified by :
Dr.Duaa Mohamed

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04/20/2026

MDT FORM

Name:	Amira Abdeltawab Aly .	MRN:	253
Age:	40 Y	ID:	26609182101265
Menopausal Sta	PRE	Phone:	012806630167
NRS SCORE:		Marital Status:	
BMI:		Family History:	-VE
CO Morbidity:	M.FREE	Reason Of MDT:	POT OP

Clinical Data:

BX: IDC G2

ER: 8/8/ **PR:** 5/8 **HER2:** 3+ **KI67:** 20%

R

L

Sonomammography:

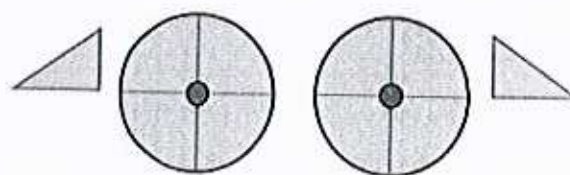
SP LT MRM POST NACT
DCIS ABSENT
NIPPLE&AREOLA FREE
DEEP MARGIN FREE
AXILLARY LN (0\14)
PATHOLOGICA; COMPLETE RESPONSE

Mets Workup:

INITIALLY T4B N2

History:**Recommendations:****MDT Outcome & Plan**

ADJUVANT HERCEPTAN (NO ANTI HER2 received as NACT)
ZOLADEX +AI
PORT



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نرمن مصطفى					



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

المنشأة المصرية
صحة
مصر

MDT FORM

26/5/2025 G C

Name	أ.م.د. عبد الوهاب	MRN	1034/2025
Age	38	ID	517.9121110
Menopausal status	pre-menopausal	Phone	0122.5061968
NRS SCORE		Marital status	Married
BMI		Family history	ve
CO morbidity	NAD	REASON OF MDT	New Case

Clinical Data

BX: IDC of Follicular DCIS

ER: 8/8 PR: 5/8 HER 2: 1.3 KI67: 25%

Sonomammography: area of Distortion
e' no Distinctive mass at 2 o'clock
e' Prominent Lt Axillary Lvs

Mets Workup:
Met workup:- free

R

L



TNM

Recommendations:

MDT OUTCOME & PLAN

BS
New SM
ACX4 + 4 TCHP

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name : الأستاذة / اميرة عبد الثواب على سرمد	Request No. : 13003820
Patient No. : 28609182101265	Request Date : 19/02/2025
Path No. : 726/25	Verification Date : 05/03/2025
Sex : Female	Financial Category : أورام دار السلام - مبانى
Age : 38 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast area of distortion.

Nature of Specimen: Tru-cut biopsy, collected at 18/2/2025.

Gross Examination:

- 3 grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for Immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal DCIS (comedo pattern).
- * Perineural invasion: No.
- * Angiolinvasion: No.
- * There is dystrophic calcification.
- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8 /8).
- PR: Strong nuclear reaction in 9% of tumor cells (score 5 /8).
- Her2: Strong complete membrane reaction in > 10% of tumor cells (score 3+).
- Ki67: 25% nuclear reaction of tumor cells.

Diagnosis:

Left breast area of distortion, Tru-cut biopsy:
Invasive duct carcinoma, Grade II and focal DCIS.

IPT:

- ER: Positive (score 8 /8).

Verified by :

DR.Menna Nabil

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Patient Name : الأمينة / اميره عبد التواب على سيد	Request No. : 13003820
Patient No. : 28609182101265	Request Date : 19/02/2025
Path No. : 726/25	Verification Date : 05/03/2025
Sex : Female	Financial Category : مبلارة - اورام دار السلام
Age : 38 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- PR: Positive (score 5 /8).
- Her2: Positive (score 3+).
- Ki67: Positive In 25% of tumor cells.

*** Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

*** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Lubna Omar.

Dr. Menna Nabil.

Verified by :

DR.Menna Nabil

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Patient Name: Amira Abd Al Tawwab Ali	Date : 29/06/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 38 Year(s)
Medical ID: 1034/2025	Pat. ID: 99267

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

- **Breast density:** Scattered areas of glandular tissue in both breasts... **Matching ACR b.**
- **A large area of distortion with extensive pleomorphic calcifications is seen in the left upper outer quadrant over an area of 6 cm ,another small grouped pleomorphic calcifications is seen in the left retroareolar region (distance from the area of distortion by 3 cm)..**
- No detected definite suspicious mass density, or pathological micro-calcification in the right breast.
- Normal skin and nipple the right side .

By complementary breast ultrasound:

- Homogenous (predominantly glandular) back ground parenchymal echo-texture.
- Thickened left breast skin (6 mm)
- An irregular shaped mass is seen at 1 o'clock position of the left breast , zone 1-2 , measuring 15 x 13 mm ,another similar mass is seen at 1 o'clock ,zone 1-2 measuring (11 x 10 mm).
- An area of parenchymal heterogeneity & distortion is seen in-between 12-2 o'clock position of the left breast zones 2..
- Multiple(more than 3 in number) suspicious looking left axillary lymph nodes ,level 1 & 2,the largest measures (17 x 17 mm) with thick cortex of 7mm .

CONCLUSION:

- Left breast : locally advanced UOQ multifocal suspicious masses and area of distortion reaching the nipple areolar complex (**BI-RADS 6 category**) .
- Right breast : normal study : (**BI-RADS 1 category**)

MUCH OBLIGED

DR.MONA ZAKY, MD.

Professor of diagnostic radiology -Breast Imaging-Mansoura University

Signature Dr. Mona Zaki

29/06/2025 2:50 PM



Patient Name: Amlra Abd Al Tawwab Ali	Date : 03/05/2026
Exam : C.T. CHEST	Age : 38 Year(s)
Medical ID: 1034/2026	Pat. ID: 89267

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of left breast cancer.

I Chest:

- Left breast retroareolar asymmetry with diffuse skin thickening.
- Enlarged left axillary level I and II lymph nodes the largest measures 8 x13 mm.
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- The liver is enlarged showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- No Intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.

ECHO REPORT

NAME : أميرة عبد التواب علي DATE: 3 / 11 / 2025
ID NUMBER : ١٠٣٤ / ٩٥ SEX : ♀ AGE: 38

EF	68%	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.3	3-3.7cm
LVEDD	2.3	2-4cm	LA	3.7	2-4cm
FS	28%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal
LV Wall Thickness Normal
LV Diastolic Function Normal
SWMA No RWMA
LA and Ao Root Dimensions Normal
Mitral Valve N
Aortic Valve N
Tricuspid Valve N
Pulmonary Valve
Right Side and Systolic Function TAPSE: 2.5
Intracardiac Masses and Effusion

DR. 



أمية في الطب

Dear Prof

Mrs Amira is a case of ER+/HER⁺
left DC CTAB No 10,
received NACT + phos go →
MRH (ypt 0 No) - she developed
Heart Failure (with recent
pleurisy CT showing mild pleural
effusion). In view of MRH
done & in view of cardiac
risk the left sided Rth & in
view of the Bst study results,
we prefer to omit PORT.